**Mohd. Amir Ansari**

Age / Sex : 19 YRS / M

Referred by : INDRA KIDNEY CARE & SURGICAL CENTER

Reg. no. : 6977

6977

Registered on : 20/10/2026 02:34 PM

Collected on : 20/10/2026 01:30 PM

Received on : 20/10/2026 01:35 PM

Reported on : 20/10/2026 02:34 PM

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## HAEMATOLOGY

### COMPLETE BLOOD COUNT (CBC)

| TEST | | VALUE | UNIT | REFERENCE |
|---|---|---|---|---|
| HEMOGLOBIN | | 13.0 | g/dl | 13 - 17 |
| TOTAL LEUKOCYTE COUNT | | 9,000 | cumm | 4,800 - 10,800 |
| DIFFERENTIAL LEUCOCYTE COUNT | | | | |
| NEUTROPHILS | H | 81 | % | 40 - 80 |
| LYMPHOCYTE | L | 16 | % | 20 - 40 |
| EOSINOPHILS | | 01 | % | 1 - 6 |
| MONOCYTES | | 02 | % | 2 - 10 |
| BASOPHILS | | 00 | % | < 2 |
| PLATELET COUNT | L | 1.35 | lakhs/cumm | 1.5 - 4.1 |
| TOTAL RBC COUNT | L | 3.60 | million/cumm | 4.5 - 5.5 |
| HEMATOCRIT VALUE, HCT | L | 33.9 | % | 40 - 50 |
| MEAN CORPUSCULAR VOLUME, MCV | | 94.2 | fL | 83 - 101 |
| MEAN CELL HAEMOGLOBIN, MCH | H | 36.1 | Pg | 27 - 32 |
| MEAN CELL HAEMOGLOBIN CON, MCHC | H | 38.3 | g/dl | 31.5 - 34.5 |
| MEAN PLATELET VOLUME, MPV | H | 12.1 | fL | 6.5 - 12 |
| R.D.W. - SD | H | 46.7 | fL | 39 - 46 |
| R.D.W. - CV | H | 14.4 | % | 11.6 - 14 |
| P-LCR | | 41.4 | | 19.7 - 42.4 |
| P.D.W. | H | 17.0 | fL | 9.6 - 15.2 |

Possible causes of abnormal parameters:

| | High | Low |
|---|---|---|
| **RBC, Hb, or HCT** | Dehydration, polycythemia, shock, chronic hypoxia | Anemia, thalassemia, and other hemoglobinopathies |
| **MCV** | Macrocytic anemia, liver disease | Microcytic anemia |
| **WBC** | Acute stress, infection, malignancies | Sepsis, marrow hypoplasia |
| **Platelets** | Risk of thrombosis | Risk of bleeding |

## SEROLOGY & IMMUNOLOGY

### VIRAL MARKER

| TEST | VALUE | UNIT | REFERENCE |
|---|---|---|---|
| HIV (CARD TEST) | | | |
| HIV - 1 | NEGATIVE | | |
| HIV - 2 | NEGATIVE | | |
| HEPATITIS C VIRUS, HCV | NEGATIVE | | |
| HBSAG | NEGATIVE | | |

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation
Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation
A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

~~~ End of report ~~~

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